

THE NUTRITION CLUB CLINIC

PCOS Guide

An introduction to nutrition and lifestyle support for hormonal and metabolic health in PCOS.

Personalised nutrition needs vary. Seek professional care for your individual situation.

What's inside

Polycystic ovary syndrome affects roughly one in ten women of reproductive age. It is a hormonal and metabolic condition, not simply an ovarian one, and nutrition and lifestyle are first-line management in every major guideline. This guide explains the drivers behind the symptoms and what helps.

1. Understanding PCOS
2. Insulin resistance: the central mechanism
3. How to eat for PCOS
4. Movement, sleep and stress
5. Supplements, medicines and follow-up

How to use this guide

Read section 2 to understand insulin resistance — it explains most of the nutrition advice that follows. Then choose changes from sections 3 to 5 that fit your life. Cycles typically take three to six months to respond, so judge progress over months, not weeks.

1. Understanding PCOS

PCOS is diagnosed using the Rotterdam criteria — **two of the following three**, with other causes excluded:

- 1 Irregular or absent ovulation (irregular, infrequent or missing periods).
- 2 Clinical or biochemical signs of excess androgens (acne, excess hair growth, scalp hair thinning, or raised testosterone on blood tests).
- 3 Polycystic ovarian morphology on ultrasound — many small follicles. Note these are follicles, not true cysts.

Common features

Area	What you may notice
Cycle	Periods longer than 35 days apart, fewer than 8 a year, or absent; difficulty conceiving
Skin and hair	Acne along the jawline, hirsutism, hair thinning at the crown, dark velvety skin patches (acanthosis nigricans)
Metabolic	Weight gain especially around the middle, strong carbohydrate cravings, fatigue after meals
Mood and sleep	Anxiety, low mood, disturbed sleep; higher rates of sleep apnoea

Why it matters long term

PCOS raises the lifetime risk of type 2 diabetes, gestational diabetes, fatty liver, high blood pressure and cardiovascular disease, and — where periods are very infrequent — of endometrial changes. Managing it is about long-term health, not only symptoms today.

You can have PCOS at any body size

Lean PCOS is common and often under-recognised. Insulin resistance can be present at a normal weight, so the nutrition principles here apply regardless of your size — weight loss is not the whole answer, and for many it isn't the answer at all.

2. Insulin resistance: the central mechanism

Up to 70–80% of women with PCOS have some degree of insulin resistance. Understanding the loop makes the rest of this guide obvious.

- 1 Cells respond poorly to insulin, so the pancreas produces more of it.
- 2 High insulin stimulates the ovaries to produce more testosterone.
- 3 High insulin also lowers SHBG, the protein that binds testosterone — so more free, active testosterone circulates.
- 4 Excess androgens disrupt follicle maturation, so ovulation becomes irregular or stops.
- 5 High insulin also promotes fat storage around the abdomen, which further worsens insulin resistance.

This is why improving insulin sensitivity — through food composition, movement, sleep and sometimes medication — tends to improve cycles, skin and energy together.

Also worth checking

- **Thyroid function** — hypothyroidism mimics and worsens PCOS symptoms.
- **Vitamin D** — deficiency is common and linked with insulin resistance.
- **Vitamin B12** — if you take metformin long term, it can lower B12.
- **Ferritin and iron** — especially with heavy bleeding.
- **Lipids, HbA1c, liver enzymes and blood pressure** — baseline metabolic screening.

Chronic inflammation

Low-grade inflammation is common in PCOS and feeds insulin resistance. An anti-inflammatory eating pattern — colourful plants, oily fish, olive oil, nuts, minimal ultra-processed food — addresses both at once.

3. How to eat for PCOS

There is no single 'PCOS diet'. What works is a pattern that keeps insulin steady and inflammation low, and that you can actually sustain.

Core principles

- 1 Build every meal around protein.** 25–40 g per meal from eggs, fish, chicken, yoghurt, pulses, tofu or paneer. Protein blunts the glucose rise and controls appetite.
- 2 Choose slow, high-fibre carbohydrates.** Whole grains, pulses, fruit and vegetables rather than white bread, sugary drinks and sweets. Don't eliminate carbohydrate — very low carb diets are rarely sustainable and can stress the cycle.
- 3 Never eat naked carbs.** Always pair a carbohydrate with protein, fat or fibre — fruit with nuts, toast with eggs, rice with dal and salad.
- 4 Include anti-inflammatory fats.** Olive oil, nuts, seeds, avocado, and oily fish two or three times a week.
- 5 Eat at regular times.** Erratic eating and long gaps followed by large meals produce bigger insulin swings and worse cravings.
- 6 Mind the drinks.** Sugary drinks, sweetened coffees and juices are pure liquid insulin load.

A sample day

Meal	Example
Breakfast	Greek yoghurt with berries, chia and walnuts; or a vegetable omelette with one slice of whole grain toast
Lunch	Chickpea and vegetable salad with olive oil and feta, plus a small portion of quinoa or brown rice
Snack	Apple with almond butter; or roasted chana; or hummus with vegetable sticks
Dinner	Grilled salmon or chicken, half a plate of vegetables, small portion of sweet potato or brown rice

If weight loss is part of your plan

Even a 5–10% reduction can restore ovulation for many women with a higher body weight. But aggressive dieting backfires in PCOS — it raises cortisol, worsens cravings and often ends in cycles of restriction. Slow, moderate and protein-rich beats extreme every time. Disordered eating is more common in PCOS, so if food feels out of control, seek support rather than a stricter plan.

4. Movement, sleep and stress

Exercise — aim for muscle

- **Resistance training 2–3 times a week** is especially valuable: muscle acts as a glucose sink and directly improves insulin sensitivity, often without weight change.
- **150 minutes of moderate cardio a week** — brisk walking, cycling, swimming, dancing.
- **Post-meal walks of 10–15 minutes** reduce the glucose and insulin spike.
- **Avoid over-training.** Excessive high-intensity exercise with under-eating raises cortisol and can suppress cycles further.

Sleep

Poor sleep worsens insulin resistance, appetite and androgen levels. Target 7–9 hours with a consistent schedule. Sleep apnoea is significantly more common in PCOS — if you snore heavily, wake unrefreshed or feel sleepy during the day, ask for a sleep assessment.

Stress and cortisol

Chronic stress raises cortisol, which raises blood glucose and insulin and can disrupt ovulation. Whatever genuinely lowers your stress — walking outdoors, breathing practice, prayer, yoga, therapy, time with people you like — is legitimate PCOS treatment, not an optional extra.

Be kind about the pace

The menstrual cycle takes roughly three months to reflect changes because follicles mature over about 90 days. Skin often takes a similar time. Judge your efforts over three to six months.

5. Supplements, medicines and follow-up

Discuss all of these with your clinician — this is information, not a recommendation.

Option	Evidence and notes
Inositol (myo- + D-chiro, 40:1)	Among the best-studied supplements in PCOS; may improve insulin sensitivity, cycle regularity and ovulation. Generally well tolerated
Vitamin D	Correct a deficiency if blood tests show one; supports insulin sensitivity and bone health
Omega-3	May reduce triglycerides and inflammation; oily fish is the food-first option
Metformin	Prescription medicine improving insulin sensitivity; can help cycles and weight. May lower B12
Combined oral contraceptives	Prescription option to regulate bleeding, protect the endometrium and improve acne and hirsutism
Letrozole / clomiphene	Prescribed ovulation induction for those trying to conceive — specialist care

Be sceptical of

Detox teas, 'hormone balancing' blends, seed cycling and expensive proprietary protocols. They are poorly evidenced and often expensive. Spend that money on vegetables, protein and a good pair of walking shoes.

Keep up your monitoring

- Blood pressure, HbA1c or glucose and lipids — typically every 1–2 years, more often if abnormal.
- A discussion about endometrial protection if you have fewer than four periods a year.
- Screening for anxiety and depression — both are more common in PCOS and treatable.
- Pre-conception review before trying to conceive, and early gestational diabetes screening in pregnancy.

The headline

PCOS is a lifelong condition, but it is highly modifiable. Consistent, unglamorous habits — protein at every meal, fibre, resistance training, sleep and stress care — change the hormonal picture more than any single supplement on the shelf.

Important note

This guide is general nutrition education only. It is not medical advice, a diagnosis, or a treatment plan, and it cannot account for your personal history, laboratory results, allergies or medicines. Always speak to your doctor, dietitian or pharmacist before making significant changes to your diet, supplements or activity — especially if you are pregnant or breastfeeding, have kidney, liver or heart disease, an eating disorder, or take prescription medication. If you experience severe or sudden symptoms, seek urgent medical care. If you are trying to conceive, have irregular or absent periods, or are considering supplements such as inositol, discuss them with your gynaecologist or endocrinologist first.

Questions or want a personalised plan?

This guide covers general principles. A one-to-one consultation with The Nutrition Club Clinic lets us tailor portions, meal timing, food preferences, budget and any medications or conditions to you.

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